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No. 96-1793

In The
Supreme Court of the United States
October Term, 1997

Cedar Rapids Community School District,
Petitioner,

v.

Garret F., a Minor By His Mother and
Next Friend, Charlene F.,
Respondent.

**On Writ of Certiorari to the
United States Court of Appeals
For the Eighth Circuit**

BRIEF OF AMICI CURIAE AMERICAN ACADEMY OF PEDIATRICS, THE NATIONAL ASSOCIATION OF SCHOOL NURSES, AND FAMILY VOICES IN SUPPORT OF RESPONDENT

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INTEREST OF AMICI CURIAE[1]

The American Academy of Pediatrics is a non-profit charitable and educational organization of 53,000 primary care pediatricians, pediatric medical subspecialists, and pediatric surgical specialists dedicated to the health, safety, and well-being of infants, children, adolescents, and young adults. Formed in the 1930s, it now has 66 chapters in the United States, its territories and Canada. The Academy is dedicated to the principle of a meaningful and healthy life for every child. It seeks to promote this goal by encouraging its members in their efforts to meet the overall health needs of children and youth; by providing support and counsel to others concerned with the well-being of children, their growth and development; and by serving as an advocate for children and their families within the community at large.

The Academy has participated as an amicus in cases before this Court and others involving issues related to the health of minors. The Academy regards this as a case in which it can make a significant contribution to the Court's consideration because of the Academy's understanding of the needs of Garret F. and similar children in a school setting. The Academy, through its committees, has studied issues relating to chronically ill children in schools for years. *See, e.g.*, American Academy of Pediatrics Committee on Children with Disabilities, *Children With Health Impairments in Schools*, 86 Pediatrics 636 (1990); American Academy of Pediatrics Committee on School Health, *Qualifications and Utilization of Nursing Personnel Delivering Health Services in Schools*, 79 Pediatrics 647 (1987).

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[1]The parties have consented to the submission of this brief. Their letters of consent have been filed with the Clerk of this Court. Pursuant to Rule 37.6, none of the parties authored this brief in whole or in part and no one other than amici, their members, or counsel contributed money or services to the preparation or submission of this brief.

The National Association of School Nurses ("NASN") is a non-profit, specialty practice nursing organization established in 1979, with approximately 10,000 members, which represents registered professional school nurses. NASN is committed to promoting quality health services, appropriate health education, and safe environments for all school-age children and youth. School nurses play an integral role in the delivery of school health services to students under the Individuals with Disabilities Education Act ("IDEA"), including assisting in identifying children who may need special education for health-related services, developing individualized health and emergency care plans, assisting in the development of the Individualized Education Program ("IEP") for eligible students, and providing and/or supervising assistive personnel to provide health services in the schooi setting. NASN, as the nation's only organization devoted exclusively to representing the interests of professional school nurses who provide school health services to students with disabilities under IDEA, has a direct interest in the outcome of the case and the Supreme Court's interpretation of the scope of IDEA "related services" and "medical services."

Family Voices is a national, charitable and educational, grassroots organization with more than 16,000 members, founded in late 1992 to speak on behalf of the 12.6 million children in this country who have special health care needs. Most of its members are families who have children with disabilities and/or chronic health conditions. However, many members are health professionals, social workers, researchers, policymakers, state and federal health agency officials and staff, or friends and relatives who also care for and about children with special health care needs. Family Voices focuses on developing and distributing objective, family-friendly materials to families to help them make informed health and other decisions on behalf of their children. Family Voices also advises

families who are negotiating with their school districts about the presence and quality of the health supports their children require in order to attend school. Family Voices, therefore, has an interest in the outcome of this case and in the responsibility of the states to assure adequate supports to students with complex health status.

STATEMENT

Garret F. is an intelligent and motivated boy who wants to continue to join other children his age in enjoying free access to education in America's public school system. *See* Pet. App. 19a. Garret is a special education student, Pet. App. 10a, who is able to attend school in the classroom, and has been attending school since 1988 when he began kindergarten. Pet. App. 2a. But because he is paralyzed from the neck down and ventilator-dependent, he needs certain caretaking tasks performed during school hours in order to attend school. *Id.* These tasks are supportive in nature and permit Garret and his mechanical equipment to function in the school setting and to participate in the educational program with other children. This support is also needed outside the school setting, twenty-four hours a day to permit Garret to live. With it, he is able to be part of his community -- to visit friends and neighbors, take vacations with his family, go on shopping trips, and take part in other activities. Garret's parents, and sometimes his friends, provide these services at home, on weekdays after school until 11:00 p.m. and on weekends without the assistance of health care personnel. Pet. App. 21a; J.A. 3-4, 19-20.

The maintenance procedures Garret requires during the school day are clean intermittent catheterization (approximately once a day), periodic suctioning (cleaning) of his tracheostomy, ventilation by means of an Ambu bag during suctioning, and monitoring his breathing and ventilator in case of malfunction. Pet. App. 24a. It is undisputed that none of these tasks needs to be performed by a physician. Pet. App. 6a, 12a, 15a. It is

also undisputed that without these services, Garret would be unable to attend school. *Id.* at 6a.[2]

Recent medical and surgical advances have markedly improved the outcomes for chronically ill children, and increasing numbers of these children survive into adulthood and are able to attend school and pursue their educational and social development. American Academy of Pediatrics Committee on Children with Disabilities, *Children With Health Impairments in Schools*, 86 Pediatrics 636, 636 (1990). More specifically, increasing numbers of accident victims with serious spinal cord injuries, like Garret, now survive but remain technology-dependent. Marilyn D. Miller et al., *Ventilator-Assisted Youth: Appraisal and Nursing Care*, 25 J. Neuroscience Nursing 287, 287 (1993). The equipment these children need has become smaller and more portable, providing the children with a new level of mobility. *See* Donna H. Lehr and Pat McDaid, *Opening the Door Further: Integrating Students With Complex Health Care Needs*, 25 Focus On Exceptional Children 1, 3 (1993); Nathan Essex et al., *Handle With Care*, 181 Amer. Sch. Bd. J. 50, 50 (1994).

The number of these children remains small, perhaps one-tenth to two-tenths of one percent of children require medical technology on a long-term basis, with feeding tubes, intravenous lines, clean intermittent catheterization, ostomy appliances and respiratory devices, such as tracheostomy tubes, ventilators, and oxygen tanks, among "the most common"

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[2]Garret also requires transportation to and from school, and help to eat and drink, to move about the building, and to turn pages. Petitioner does not challenge its obligation to provide these services, and has provided Garret with a full-time Teacher Associate ("TA"). Pet. App. 27a, 28a. Further, petitioner has stipulated that the person who performs the disputed services could also perform the TA's duties. *Id.*; *see also* Pet. Br. at 6.

technologies. Stephanie Porter et al., *Children and Youth Assisted by Medical Technology in Educational Setting - Guidelines for Care* 4 (1997). Over the past twenty years, "more and more technology-dependent children have entered the mainstream school system." Genie L. Wessel, et al., *School Placement and the Oxygen-Dependent Child*, 4 J. Pediatric Nursing 435, 435 (1989). "[T]here has been an influx of technology-dependent children into schools, and the child requiring oxygen therapy is frequently seen." *Id.*[3]

States and schools throughout the country have demonstrated their willingness and ability to enroll children with serious (and even life-threatening) illnesses such as muscular dystrophy with respiratory compromise, terminal cancer, HIV-AIDS, and end-stage renal disease. These children require and receive a wide variety of related services, enabling them to attend public schools with their peers.

Ventilator-dependent children who are stable, like Garret, can be integrated into their communities, and participate with other children in school and after school activities, if they are provided the maintenance services required. *See, e.g.*, Miller et al., *supra*, at 294-95 (reporting study of 16 ventilator-assisted, cervical-cord injured youth, aged 3 to 19, finding that 13 lived

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[3]*See also* Lehr and McDaid, *supra*, at 2 ("the numbers of students with complex health care needs who are being educated in schools are continually increasing" for two reasons: "[c]hanges in medical technology are enabling children with medical complications to survive" and the "generally increasing practice of normalizing lives for student[s] through the specific practice of including students with more severe disabilities into general education settings"); B.J. Make et al., *Mechanical Ventilation Beyond the Intensive Care Unit, Report of a Consensus Conference of the American College of Chest Physicians*, 113 Chest 289S, 327S (1998 Supp.) ("Many ventilator-assisted children have been successfully attending school for years.").

at home and attended school in classroom settings, with staff at the school trained to meet their specific needs, and that their extracurricular activities included church, 4-H, summer camp, trips to Disney World and family vacations).

The development of modern advances in medications, portable ventilators, non-invasive ventilation techniques, suction machines, wheelchairs with sliding trays for ventilators, effective speaking valves, accessible public and private buildings and transportation makes it possible for ventilator-dependent adults to hold jobs and enjoy social functions and friendships. *See, e.g.*, Tedde Scharf, *Yes, There Is Life After Ventilation*, 103 Chest 1319 (1993). Attending school can prepare children like Garret to take advantage of these "doors to unlimited opportunities [that] have opened" and to develop the intellectual and social skills needed to live "a fully functional, independent existence." *Id.*

SUMMARY OF ARGUMENT

The services at issue here are not treatment. They are not provided by a doctor or hospital. They are simply services necessary to operate and monitor the equipment that substitutes for the functions Garret has lost in order to keep him alive. With that equipment, he is able to be with family and friends and to acquire, from the public school system, the educational and social skills he will need to participate in society initially as a child and later as an adult.

It is undisputed that Garret is capable of attending public school and benefiting from the educational and other opportunities provided by the public school system; he has been doing so for years. The only dispute is whether the Iowa Department of Education's specific finding -- that the monitoring and maintenance services Garret requires while he attends school must be provided, *see* Pet. App. 63a -- is consistent with the statute Congress enacted to encourage states to provide children with disabilities access to public education and consistent with the United States Department of Education's regulations implementing that statute. Because the state agency's decision is consistent with both, *amici* urge this Court to affirm.

1. Congress enacted the Individuals with Disabilities Education Act in order to encourage states to provide public school educations for children with disabilities. Congress predicated federal assistance for educating such children on the states' providing not only special education when needed but also related support services to enable the children to benefit from the education, excepting only medical services other than diagnosis and evaluation. Pt. I.A.

2. The Secretary of Education's implementing regulations -- requiring states to provide nursing services but exempting them from providing services performed by a physician -- are permissible under the statute and therefore due deference under *Chevron*. Pt. I.B.

3. Petitioner's proposed balancing test for differently determining which services are exempt is merely an alternative policy choice. It has no basis in the statute, and is, indeed, *inconsistent* with the statute, as Congress made clear when it prohibited the agency from decreasing required support services below 1982 levels, without unequivocal support from subsequent legislation. In addition, the test petitioner proposes has the practical infirmity of being difficult to apply and to police. Pt. I.C & Pt. I.D.

4. The Iowa Department of Education properly applied the statute and regulations in finding that Garret must be provided the maintenance and monitoring services he requires to attend school. Pt. II.

5. Contrary to petitioner's complaints, the federal government is *not* forcing high costs on school districts. The federal statute and regulations offer states federal assistance if they agree to meet set standards, including providing support services to children with disabilities. States are required to comply *only if* they choose to avail themselves of the assistance. Congress delegated decisions as to the sources of additional funding to the states, so long as there is no charge to the family. Thus, it is the State of Iowa's decision whether the school district, a state agency, the family's insurance, or some combination funds the services Garret requires. Pt. III.

ARGUMENT

I. CONGRESS INTENDED FOR CHILDREN LIKE GARRET TO ATTEND PUBLIC SCHOOLS, AND THE AGENCY'S REGULATIONS FAITHFULLY IMPLEMENT THAT GOAL.

A. The Statute

More than two decades ago, Congress recognized the need to guarantee children with disabilities like Garret a free and appropriate education, and to do so, Congress created significant incentives for states and communities to provide public education for children with disabilities in the Education for All Handicapped Children Act of 1975,[4] now known as the Individuals with Disabilities Education Act ("IDEA").[5] Congress found that there were, in 1975, more than eight million children with disabilities in the United States and that the educational needs of more than half of these children were not being met. 20 U.S.C. § 1400(b)(1) & (2). Congress further found that one million of these children were being excluded entirely from the public schools, and the opportunity to go through school with their peers, because of the lack of adequate services within the public school system. *Id.* at § 1400(b)(4) &

[4] Pub. L. No. 94-142, 89 Stat. 773.

[5] *See* 20 U.S.C. § 1400 *et seq*. The statute was renamed in 1990, as part of the Education of the Handicapped Act Amendments of 1990, Pub. L. No. 101-476, 104 Stat 1103. In 1997, Congress expanded the statute's definition of "related services" to include "orientation and mobility services," and renumbered certain section of the Act. Individuals with Disabilities Education Act Amendments of 1997, Pub. L. No. 105-17, § 101, 111 Stat. 45 (codified at 20 U.S.C. § 1401(a)(22)). Citations to the statute in this brief are to the 1994 code in effect when respondent initially sought support services.

(6). The IDEA was enacted in order "to assure that all handicapped children have available to them . . . a free appropriate public education which emphasizes special education and related services designed to meet their unique needs, to assure that the rights of handicapped children and their parents or guardians are protected." *Id.* at § 1400(c).[6]

In order to receive federal funds to assist in providing such an education, states must assure all children with disabilities, regardless of the severity of their disabilities, the right to a free appropriate public education, *Id.* at § 1412(1), (2)(B)-(C), which is defined to include both "special education" and "related services" at public expense and without charge. *Id.* at § 1401(a)(18). The statute defines related services as:

> transportation, and such developmental, corrective, and *other support services* (including speech pathology and audiology, psychological services, physical and occupational therapy, recreation, including therapeutic recreation, social work services, counseling services, including rehabilitation counseling, and medical services, *except that such medical services shall be for diagnostic and evaluation purposes only*) as may be required to assist a child with a disability to benefit from special education....

[6] It is also amici's view that school attendance is preferable to homebound teaching and should be encouraged whenever possible. Homebound teaching has significant limitations, in addition to the isolation of children with disabilities from their peers. It consists typically of only a few hours per week, with a single teacher expected to cover all the subjects the child is studying. *See* American Academy of Pediatrics, *Children With Health Impairments in Schools*, 86 Pediatrics 636, 637 (1990).

20 U.S.C. § 1401(a)(17) (emphasis added).[7] As this Court has found: "the face of the statute evinces a congressional intent to bring previously excluded handicapped children into the public education systems of the States . . . ." *Board of Education of Henrick Hudson Central School District v. Rowley*, 458 U.S. 176, 189 (1982).

At issue in this case is the scope of the "medical services" exclusion to the statute, Pet. App. 11a, an issue this Court has addressed before in *Irving Indep. Sch. Dist. v. Tatro*, 468 U.S. 883 (1984). There is no dispute that the services required by Garret are related services within the meaning of the statute. As this Court found in *Tatro*, services "that enable[] a handicapped child to remain at school during the day [are] an important means of providing the child with the meaningful access to education that Congress envisioned." 468 U.S. at 891.

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[7]Contrary to petitioner's assertions, the provision does not purport to enumerate every service that must be provided, Pet. Br. at 10, but simply provides examples of some services that are included in the general category of supportive services. Thus, this Court was not constrained from finding clean intermittent catheterization to be a required supportive service, even though it is not one of the enumerated services. *See Irving Indep. Sch. Dist. v. Tatro*, 468 U.S. 883 (1984). Further, petitioner also misreads the statute to find support for its position that the statute imposes obligations to provide only educational services, Pet. Br. at 15-16. The very provision at issue expressly requires services, such as transportation, psychological services, and social work services, which are not themselves educational services but must be provided in order to permit children with disabilities to benefit from educational services.

B. The Department of Education's Implementing Regulations

The Department of Education's implementing regulations define "medical services" as "services provided by a licensed physician." 34 C.F.R. § 300.16(b)(4).[8] Further, the agency regulations include "school health services" among the "related services" that must be provided for children with disabilities, 34 C.F.R. § 300.16(a), and define "school health services" as services that can be provided by a qualified school nurse or other qualified person. 34 C.F.R. § 300.16(b)(11). This Court relied on these regulations to conclude that the Secretary had determined that the services of a school nurse "are not subject to exclusion as a 'medical service,' but that the services of a physician are excludable as such." *Tatro*, 468 U.S. at 892.[9]

Because the Department of Education is the agency entrusted to administer the statute and to issue whatever rules

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[8]Recognizing that the regulation expressly attempted to define only those medical services that are owed to children under the statute, *i.e.*, diagnostic and evaluative services, this Court concluded that the regulation meant "that 'medical services' *not* owed under the statute are those 'services by a licensed physician' that serve other purposes." *Tatro*, 468 U.S. at 892 n.10 (emphasis in original). The Secretary of Education has interpreted these regulations in the same way. See *Assistance to States for Education of Handicapped Children, Notice of Proposed Rulemaking*, 47 Fed. Reg. 33,836, 33,838 (1982).

[9]States are therefore not required to ensure that schools are prepared to provide surgical interventions as needed, such as trimming the tissue around a tracheostomy or performing an incision and drainage of an abscess, or to prescribe medications, or to administer chemotherapy or radiation therapy, or to perform hemo dialysis and transfusions, or to take x-rays, or to provide any other of the array of services provided by doctors or in hospitals.

and regulations are necessary to carry out its provisions, 20 U.S.C. § 1417(b), the Department's elucidation of the medical services exclusion controls unless it is "arbitrary, capricious, or manifestly contrary to the statute." *Chevron, U.S.A., Inc. v. Natural Resources Defense Council*, 467 U.S. 837, 844 (1984); *accord Pension Benefit Guaranty Corp. v. LTV Corp.*, 496 U.S. 633, 647-48 (1990). The Department's distinction between services provided by nurses and services provided by doctors or hospitals suffers from none of these infirmities. As this Court noted in *Tatro*, "Congress plainly required schools to hire various specially trained personnel to help handicapped children," and "[s]chool nurses have long been a part of the educational system," while services provided by physicians and hospitals "might well prove unduly expensive and beyond the range of [a school system's] competence." 468 U.S. at 892-93. Thus, this Court determined that "limiting the 'medical services' exclusion to the services of a physician or hospital" was a "permissible construction of the provision" and "entitled to deference." *Id.* at 892-93 (finding clean intermittent catheterization -- one of the services at issue in this case -- a service schools must provide to children who require it in order to attend school).[10]

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[10]Petitioner's claim that the Department's position has changed since *Tatro* has already been contradicted by the government itself. Gov. Br. (*opposing certiorari*) at 8. Moreover, the one letter from anyone in the Department that petitioner chose to submit, Pet. App. 64a-67a, does not evidence any change in the Department's position nor does it purport to interpret the Department's regulations, which are simply spelled out. Pet. App. 66a, n.1. What petitioner attempts to rely on is the program officer's attempt to summarize existing lower court decisions and his abstention from general advice in light of them. Pet. App. 66a-67a. Advice that takes into account the power of courts to set aside the agency's position has no bearing on whether the agency has, in fact, changed its position. What is relevant is that the regulations at issue have not been altered since they were interpreted by both the Department and this Court to draw a bright line between services provided by nurses and services provided by physicians and hospitals.

That this Court's conclusion in *Tatro* is equally applicable when those same regulations are applied to children like Garret is supported by Congress' express intention to provide education to children with severe, as well as moderate, disabilities and to "mainstream" children with disabilities, placing them with children without disabilities, to the extent possible. Thus, Congress made clear that its goal was to reach all children with disabilities "regardless of the severity of their handicap," 20 U.S.C. § 1412(2)(C), and Congress required states, as a condition to receiving federal funds, to give priority to children with disabilities who were not receiving an education and children "with the most severe handicaps who are receiving an inadequate education." *Id.* at § 1412(3).

Congress also expressly provided that children with disabilities be educated with other children "to the maximum extent appropriate" and that "removal of handicapped children from the regular educational environment occurs only when the nature or severity of the handicap is such that education in regular classes *with the use of supplementary aids and services* cannot be achieved satisfactorily." *Id.* at § 1412(5)(B) (emphasis added). In enumerating some of the supplementary services that schools receiving federal aid were required to provide, *id.* at § 1401(a)(17), Congress named services that require schools to hire specially trained personnel, *Tatro*, 468 U.S. at 893, and that are considerably more pro-active than the support services at issue here. For example, physical and occupational therapy, speech pathology, and audiology all require specialized health care personnel and are aimed at improving the health of the children receiving these services rather than simply maintaining their current health status.

Congress' express intentions -- to reach children with severe disabilities, to include children in regular classrooms whenever supplementary services can make that possible, and to require schools to hire specialists when needed to provide supplementary services -- makes clear that requiring schools to provide services that simply maintain a child while in school so that he can participate in the classroom is wholly consistent with Congress' goals. The Department of Education's implementation of the statute to require states to provide such supplementary nursing services is, therefore, a permissible construction to which this Court should defer.

Petitioner's argument that the "medical services" exclusion is intended to exclude all health care services other than diagnosis and evaluation -- and that therefore the distinction between nurse-provided services and physician-provided services is impermissible -- is contrary to *Tatro*'s specific finding with respect to clean intermittent catheterization and plainly wrong on its face. *See* Pet. Br. at 14-15. Congress' express language contradicts petitioner's claim that Congress meant the scope of "medical services" in Section 1401(a)(17) to be co-extensive with the scope of "medical assistance" in Title XIX of the Social Security Act (Medicaid). In defining "related services" in Section 1401(a)(17), Congress made clear that physical therapy, speech therapy and occupational therapy must be provided, but that no "medical services" need to be provided other than diagnosis and evaluation. Thus, whatever the phrase "medical services" was intended to cover, it did not include physical therapy, speech therapy or occupational therapy. In contrast, the phrase "medical assistance" in Title XIX was expressly defined as including physical therapy, 42 U.S.C. § 1396(d) (1975), and interpreted by the agency to include occupational and speech therapy as well. *See American Physical Therapy Ass'n v. Prudential Life Insurance Corp.*,

502 F.2d 500, 502 (1974). Thus, the text of each statute demonstrates that Congress intended to delineate a narrower scope of services by the phrase "medical services" in Section 1401(a)(17) than it intended to describe by the phrase "medical assistance" in Title XIX.

Petitioner's reliance on the definition of "medical care" in the Internal Revenue Code is similarly unavailing because Congress intended the phrase "medical care" in the tax code to cover a much broader range of services than it intended the phrase "medical services" to cover in Section 1401(a)(17). For example, "medical care" in the tax code has long been understood to include transportation, psychological services, speech therapy, and even tuition for special schools which provide such services.[11] None of these fall within "medical services" under Section 1401(a)(17). Under Section 1401(a)(17), the phrase "medical services" covers diagnosis, evaluation, and those services Congress exempted schools from providing. By expressly requiring schools to provide transportation, psychological services, speech therapy and special education to children with disabilities, Congress makes clear that these are not "medical services" under the IDEA.[12]

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[11] *See, e.g.*, 26 U.S.C. § 213(d)(1)(B) (providing that "medical care" includes amounts paid for transportation to obtain medical care); *Fischer v. Commissioner*, 50 T.C. 164, 175 (1968) (noting Rev. Rul. 143, 1953-2 C.B. 129, finding psychological services included in "medical care"); *Greisdorf v. Commissioner*, 54 T.C. 1684 (1970) (tuition may be deducted as "medical care" when special school provides psychological services); *Weinberg v. Commissioner*, T.C. Memo 1969-003 (1969) (tuition may be deducted as "medical care" when special school provides speech therapy).

[12] Petitioner's attempt to find a discrepancy between the agency's interpretation of Section 1401(a)(17) and the text of Section 1413(e), Pet. Br. at 17, is similarly misguided. Petitioner not only ignores the fact that the phrases at issue are not identical, but also misconstrues Section 1413(e).

C. Petitioner's Alternate Policy Choice

Petitioner's proposed balancing test has no basis in the statute or the regulations, but is merely an alternate view of appropriate federal policy by an interested party. Petitioner impliedly admits this by its failure to cite any authority in support of its proposed test. *See* Pet. Br. at 11, 34. The statute distinguishes between medical services and other support services necessary to permit children with disabilities to benefit from public school education. The regulations implement the distinction as one between physician-provided services and services provided by others, including school-hired nurses. Neither the statute nor the regulations suggest that schools should, or even may, weigh the cost of the services needed or the length of time services are needed. Nor are the "factors" urged by petitioner consistent with any understanding of "medical services" within the medical community. Whatever lines may be drawn between medical and non-medical services within the profession, they are not based on the cost, duration, or technical complexity of the care involved.[13]

Moreover, the proposed balancing test is far from self-executing. Petitioner itself feels obligated to provide nursing

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Section 1413(e) simply assures children with disabilities and, indirectly, schools that funds through Medicaid and the Maternal and Child Health Program remain available as before. Congress had already made clear in the previous section that state agencies other than school districts might be required to pay for some or all of the costs necessary to provide a child with a free appropriate public education. 20 U.S.C. § 1412(6).

[13]For example, both writing a prescription and performing neurosurgery are considered medical services notwithstanding the differences in cost, duration, and technical complexity.

services to children with disabilities and, if needed, a full-time one-on-one support service provider, like the one it has made available to Garret for years. In refusing to provide one-on-one nursing services, petitioner is not drawing a line between nursing and other support services or between hiring personnel and using existing personnel or even between one-on-one services and occasional services. It is simply drawing a line between a less expensive provider of services and a more expensive provider of services.

Moreover, petitioner's proposed balancing test opens the door to the difficult task of drawing many more such lines. At what point do nursing services cost too much? When a service must be performed three times a day? Ten times a day? When a nurse must spend an hour a day with a child? Three hours a day? At what point does any other support service cost too much? Is a one-on-one interpreter for a deaf student too much, even when the student is unable to learn without an interpreter, if the hourly rate for an interpreter is higher than the hourly rate for a teaching aide?

At the least, petitioner is asking to replace a reliable standard -- the distinction between services provided by nurses and services provided by physicians and hospitals -- with an unreliable one that will inevitably generate endless administrative appeals and litigation. Further, the vagueness of the test, and its reliance on cost as a basis for denying services, can be expected to frustrate Congress' express goal of educating children with disabilities with other children in the public schools because the test will encourage school districts

to feel freer to deny needed services, resulting in fewer children with disabilities able to attend school.[14]

The nub of the District's position is a complaint about cost and, in its view, an inadequate level of federal assistance. Pet. Br. at 19. What the District disregards is that the calculus, with respect to the bargain offered by Congress, has been expressly left to the states. 20 U.S.C. § 1400 *et seq*. If the State of Iowa considered the requirements imposed on states by the statute too arduous and the federal assistance offered in return insufficient, which it plainly does not, it would have freed itself of those requirements by disengaging from the federal program.

Petitioner's real complaint is that Iowa has not done so. Indeed, Iowa has enacted legislation that *independently* requires petitioner to supply the disputed services.[15] Furthermore, Iowa is not alone. It is *amici's* understanding that every state has made the choice to participate in the federal program. It may be that these states are balancing not simply the cost of providing nursing services to students with severe disabilities but also the cost of *not* doing so -- the later costs to society and to government coffers of adults with severe disabilities who have been isolated and undereducated. In any event, it is the state's

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[14]Petitioner's reliance on *Tatro*'s discussion that Congress may have wanted to spare schools from an obligation that might prove unduly expensive and beyond the range of their competence is unavailing. As *Tatro* actually held, that goal is served by the Department's distinction between services provided by nurses and services provided by physicians. 468 U.S. at 892. It, therefore, provides no support for rejecting the Department's distinction in favor of petitioner's balancing test.

[15]*See* Pet. App. 58a (Iowa Department of Education ALJ's decision that Iowa law independently requires the necessary school health services to be provided for Garret).

choice, and the local school district's obligation to comply, whatever its policy preferences.

D. Congress' Subsequent Actions

Congress' subsequent actions strongly support the Department's definitions of "related services" and "medical services." The Department's one attempt to alter its definitions of "related services" and "medical services," to exempt school districts from having to provide the types of services Garret needs, was met with such overt hostility from Congress that the proposed regulations were withdrawn. Petitioner does not dispute this. Pet. Br. at 24-25.

In 1982, with a new administration in place, the Secretary of Education proposed issuing regulations that, among other things, would have removed "school health services" from the definition of "related services," and would have excluded, as medical services, "[l]ife-sustaining procedures that . . . [m]ust be administered by specially-trained, licensed health care professionals." *Assistance to States for Education of Handicapped Children, Notice of Proposed Rulemaking*, 47 Fed. Reg. 33,836, 33,838, 33,846 (August 4, 1982). Further, the proposed regulations would have permitted school districts to determine whether to provide services based on the level, frequency, location, and duration of the services required, and the qualifications of the service providers. *Id.* at 33,846.

Congress, as well as the public, reacted so strongly against these proposals that the Secretary quickly withdrew them. Five weeks after the Secretary issued the Notice of Proposed Rulemaking, Congress passed a resolution explaining that it was the sense of Congress that the proposed regulations should not be adopted. *See* section 305(b), Pub. L. No. 97-257, 96 Stat.

874 (September 10, 1982). Two and one-half weeks later, the Secretary withdrew its proposed changes to, among others, the definitions of "related services" and "medical services." *Assistance to States for Education of Handicapped Children, Modification of Notice of Proposed Rulemaking*, 47 Fed. Reg. 49,871, 49,872 (1982) (explaining that the Secretary had withdrawn these proposed changes on September 29, 1982 and no longer intended to adopt them). Less than a year later, Congress took the extraordinary step of restricting the power of the Secretary to alter its regulations in any manner that would lessen the protections provided to children with disabilities that were then available. *See* Pub. L. No. 98-199, § 608(b), 97 Stat. 1359 (1983), codified at 20 U.S.C. § 1407(b).[16] Thus, in 1982 and 1983, Congress enacted legislation to ensure that the agency's regulations limiting the "medical services" exclusion to physician-provided services[17] remained in place.[18]

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[16]The provision prohibits the Secretary from implementing, or publishing in final form, any regulation that "would procedurally or substantively lessen the protections provided to handicapped children [under the statute], as embodied in regulations in effect on July 20, 1983 (particularly as such protections relate to . . . least restrictive environment, related services. . .), except to the extent that such regulation reflects the clear and unequivocal intent of the Congress in legislation." *Id.*

[17]The Secretary had explained, in its NPRM, that under the existing regulations, the medical services that were excluded were services provided by a licensed physician. 47 Fed. Reg. at 33,838.

[18]Petitioner's claim that Congress failed to take affirmative steps to ensure that nursing services were understood to be required related services ignores this part of the legislative history in which Congress did precisely that. Moreover, petitioner's reliance on Congress' inaction in the face of merely lower court decisions -- when this Court's only decision on the issue, *Tatro*, upheld the Department's regulations and appeared to interpret them to require schools to provide nursing services -- disregards not only sound legislative policy but also sound judicial policy. *See, e.g., Zuber v. Allen*, 396 U.S. 168,

II. THE SERVICES GARRET REQUIRES TO ATTEND SCHOOL ARE NON-MEDICAL RELATED SERVICES STATES MUST PROVIDE.

Garret is precisely the type of child that Congress intended to assist. The services Garret needs can be provided by non-physicians. Nurses or other properly prepared, appropriately trained, and adequately supervised personnel can perform these tasks, which involve assessment, support, monitoring, and health maintenance that merely enable Garret to remain at school and participate in the educational program for the full school day. Garret's needs do not require a physician's services on site while Garret attends school. Thus, the services at issue fall within the statutory provision, as interpreted properly by the agency to which Congress delegated the substantive task of implementing its legislation.

The role of the primary care physician under these circumstances is to coordinate the health care Garret receives, both at home and at school. The overall direction of medical care and health-related services for children with chronic and disabling conditions is the responsibility of the primary care physician and the medical community, wherever the services may be provided, and whoever may provide them.[19] It is

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185-86, n.21 (1969) ("It is at best treacherous to find in [C]ongressional silence alone the adoption of a controlling rule of law.") (quotation and citation omitted).

[19]That physicians may prescribe certain services or that these services are performed under a physician's ultimate supervision does not transform services provided by a nurse or other trained health provider into "medical services" within the meaning of the statute, as this Court found in *Tatro*. *See* 468 U.S. at 893-94 (rejecting just such an argument and noting that school

important that the child's personal physician and the school physician, the school nurse, school administrator and the family develop a written health plan as an integral component of the child's Individual Educational Plan (IEP). Thus, it is the role and responsibility of a child's primary care physician to inform school health professionals about the child's medical needs and child's degree of fragility.

Working with this information, school health professionals (e.g., physicians or nurses) can decide whether the services should be provided by a nurse or delegated to an adequately trained and appropriately supervised health professional. The specific decisions with respect to whether a registered nurse, licensed practical nurse, or other health professional is required for a child must be made on a case-by-case basis and must be redetermined whenever there is a change in the child's condition.

Garret's pediatrician determined that Garret requires the following services to attend school.

1. Clean intermittent catheterization ("CIC"). CIC can be performed by a trained health aide. And it can be performed in a school bathroom or nurse's office, or anywhere the child has privacy. Stephanie Porter, *supra*, at 218. Further, this Court has previously found specifically that CIC is a service schools must provide as a "related service" under the Act. *See Tatro*, 468 U.S. at 895.

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nurses dispense medications or administer emergency injections in accordance with a physician's prescription to children without disabilities and that Congress certainly was not unwilling to guarantee children with disabilities "services of a kind that are routinely provided to the nonhandicapped").

2. Suctioning of the Tracheostomy. When first established, and for the first year or so, a tracheostomy requires careful monitoring and close supervision. This can be done by a licensed practical nurse supervised by a registered nurse or a registered nurse. As the tracheostomy track forms with scar tissue in the neck, the individualized care becomes more routine. Garret's tracheostomy has existed for several years and routine monitoring is sufficient. Suction is needed to clear away secretions or mucus. The attendant must be in sufficiently close physical proximity to see and hear Garret and his equipment.

3. Assisted ventilation by means of an Ambu bag. During suctioning, Garret may need to be given a few breaths through the use of an Ambu bag. In addition, the Ambu bag can be used in an emergency for Cardio Pulmonary Resuscitation ("CPR"). This type of CPR is significantly easier to perform than mouth to mouth resuscitation.

4. Monitoring the ventilator. Like any mechanical device, a ventilator can malfunction. Because Garret needs the ventilator to breathe, a health service provider who has been evaluated and has demonstrated competence in monitoring the machine and its function and responding appropriately when there is a malfunction should be available and close enough to see or hear the ventilator at all times. This can be done by specially trained health personnel; a physician is not required.[20]

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[20]Ventilator-dependent children may also be at risk of autonomic hyper-reflexia. Where the risk is limited, an attendant for such a child should also be able to recognize the signs and call 911 for emergency medical services if needed or, in the appropriate case, perform a clean intermittent catheterization procedure. For children at high risk of autonomic hyper-reflexia, school attendance may be contraindicated. The primary care physician must evaluate the needs of each individual child with disabilities. Garret has apparently never experienced autonomic hyper-reflexia at school, and has not experienced

Amici firmly believe that the above services are not and should not be considered medical services, because none must be provided by a physician and none require on-site supervision by a physician in the school setting. To the contrary, supportive services provided to a student assisted by a ventilator can be performed by a qualified registered nurse, respiratory therapist, or other health professional that has participated in and successfully completed a competency-based training program. *See, e.g.*, Stephanie Porter et al., *supra*, at 308. Moreover, it is not necessary that the same person perform all services.

A registered nurse, for example, would be one appropriate provider of services for Garret. However, depending upon Garret's current condition, these services may be delegated to other personnel. Delegation, of course, should be in accordance with state and local regulations, including state nurse regulations. Here, the Iowa State Board of Nursing has clearly indicated that a registered nurse may delegate the performance of Garret's supportive services to other health professionals. *See* Gov. Lodging (Letter from Lois Churchill, Associate Director, Board of Nursing, to Sue L. Seitz, Counsel of Record for Garret (June 13, 1997)).[21]

Whoever performs these daily services, whether a health professional or a personal attendant, the pediatrician or other primary care physician or supervising nurse must still remain

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this condition at all in recent years. Pet. App. 20a.

[21]The school nurse, in consultation with the child's primary care physician and medical team, should determine the competence required for the delegated personnel and arrange, train and supervise the personnel, and all such activities should be documented. In addition, the parents should be involved and understand and consent to the choices of which personnel are providing which services for their child.

ultimately responsible to evaluate which services need to be performed and how Garret is responding and to ensure that needed services are performed.[22] Delegation therefore requires coordination and collaboration between all individuals involved in Garret's care. However, none of the actual support services Garret needs to attend school with his peers need to be performed by a physician nor do they require a physician to be on the school grounds at the time they are performed.

III. CONGRESS' GOAL OF ENHANCING EDUCATION FOR CHILDREN WITH AND WITHOUT DISABILITIES WILL BE UNDERMINED IF THESE SERVICES ARE DEEMED "MEDICAL SERVICES."

Before the enactment of the IDEA, many children with special needs or life-sustaining equipment were relegated to home tutors or special education centers. Under the statute, however, the school environment has undergone an evolution with increasing numbers of children who, like Garret, are dependent on technology entering the school system. See Wessel et al., *supra*, at 435; Lehr and McDaid, *supra*, at 2, 3; Essex et al., *supra*, at 50. Notwithstanding his paralysis, Garret is not only intelligent and mentally stable but also generally in good health and able to effectively verbally communicate his needs to those around him. He will do much better,

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[22]There is also a critical need for the school authorities and the families of children with severe disabilities mutually to determine precisely when and how responsibility for the child's health will transfer from the family to the school and back to the family, including the circumstances of the transportation between the family home and the school. Similarly, specific intra-school responsibility for ventilatory and other care must be documented to ensure the child's safe transportation between, and participation in, various non-classroom in-school activities. These needs do not appear to be in dispute in this case.

educationally and socially, attending school with other children than remaining at home. His presence is not substantially disruptive to the classroom, and the teacher's time is not unduly burdened by working with him individually.

In addition, Garret's presence in the classroom enables his classmates and teachers to learn of the needs of all children and to observe directly the equitable treatment of those with special needs. The statute not only facilitates education of children with disabilities in the least restrictive environment possible, but also facilitate a different, but equally important, kind of educational exchange to occur, by helping dispel children's fear of disabilities in general. Providing every child with special health needs the opportunity to interact and participate in school activities with peers is an important a part of an appropriate education for all children. See Wessel, *supra*, at 436. Having these supports in place benefits the larger community of the school in its ability to respond to the needs of all students, faculty and staff.

With respect to the impact of the costs of these services, these costs are incurred only if states choose to avail themselves of federal funding through the IDEA. Providing access to public education for children with disabilities, according to standards established by Congress and the Department of Education, is the price Congress exacts for its considerable help in funding special needs education. Moreover, the IDEA does not mandate that the funds for support services for children like Garret come exclusively from school district funds. To the contrary, in 1986 Congress amended the statute to stress the necessity for coordination among different state agencies with respect to implementing the standards specified by the statute. *See* 20 U.S.C. §§ 1412(6), 1413(e). The source of funding for Garret's services can be school funds, federal funding and/or

state health programs (such as Medicaid, Maternal and Child Health programs, etc., which are also significantly supported by federal funding), family resources such as insurance or some combination of the above. *See, e.g.*, 34 C.F.R. § 300.301(a). Thus, the financial impact on the school district can be softened and controlled.

However, when family insurance proceeds are exhausted, states must provide the funding necessary to keep children like Garret in the same public school system that is available free of charge to children without disabilities. Although Congress provides considerable assistance to states willing to undertake these obligations, it left to the states the task of providing additional funds and of determining allocation mechanisms. Thus, whether the additional funding required here must be provided by the Cedar Rapids Community School District, a state agency, or some combination of sources is a choice delegated by Congress to the State of Iowa.

Children with disabilities deserve access to school-based educational services and to a broad range of school activities. The implementation of the IDEA has markedly improved the availability and use of related health services in schools to assist children with disabilities. To deprive Garret of this opportunity simply because his related services require more attention from qualified health care professionals than the services required by other children with disabilities would directly undermine the intent of Congress to include children with disabilities in the nation's public schools. It would run counter to the extensive progress that has been made in providing access for children with disabilities to an educational program in the least restrictive setting possible in the public school system. If the support services Garret needs to obtain access to public education are classified as "medical services" outside the statute's definition

of related services that must be provided, it will leave Garret with no choice but to be educated at home -- precisely the result Congress intended to prevent whenever possible.

CONCLUSION

For the above reasons, *amici* urge the Court to affirm.

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